

جوستاف روسي انترناسيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إِسْمُ الْمَرِيضِ رِبَاعِيًّا : ولاء كمال حنفى حنفى

المحافظة : القاهرة

الحي / المدينة : مصر القديمة

العنوان : مـــــــــــــــــــــــــــــــــم

رقم البطاقة : 28908020103245 رقم التليفون : 01143290099 تاريخ تسجيل الإستمارة : 20-09-2026

العمر(تاريخ الميلاد) : 02-08-1989 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي
التوقيع: (_____)

المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي للرقبة والصدر والبطن والحوض بالصيغة [11128]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة:

الوظيفة:

الوظيفة:

التوقيع:

التوقيع:

التوقع:

تتعمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطّة العلاج :

المستشفى :

جـهة العلاج :-

ملاحظات : ---

رئیس

المجلس الطبي المتخصص

مدير عام

مديرية الشؤون الصحية

ختم

شعار الجمهورية

--

Name: ولاء كمال حنفي حنفي National ID: 28908020103245	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of neck, chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient Name : الأستاذة / ولاء كمال حنفى حنفى على
Patient No. : 28908020103245
Path No. : 142/22
Sex : Female
Age : 32 Y
Referring Doc :

Request No. : 138551
Request Date : 13/01/2022
Verification Date : 19/01/2022
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data:
Right breast mass.

Nature of the specimen:
Right breast wide local excision & sentinel lymph nodes biopsy.

Gross Examination:

- Procedure: Right wide local excision specimen; measured 22x11x4 cm, skin ellipse 17x4.5 cm
- Localization: UOQ
- Tumor extent:
 - o Number: Three masses; 2.7x2 cm, 2x2 cm & 1x1 cm. Also a satellite nodule 1x1 cm.
 - o Skin involvement: No.
- Distance from margins: least surgical margin 0.5 cm (medial margin).
- Lymph nodes number and size: 6 nodules up to 1.5 cm.
- Additional pathologic findings: Extra medial margin 6x5.5 cm, unremarkable grossly.
- Immunohistochemistry technique: N/A
- Frozen section: Results: Free margins, free SLN (0/6).

Microscopic examination:

- Histological type:
 - Largest mass: Invasive duct carcinoma, grade II (3+2+2)
 - Second mass: Mixed Invasive duct carcinoma and mucoid carcinoma, grade II (3+2+2)
 - Third mass: ductal carcinoma insitu.
- Free margins
- free SLNs: (0/6)

Diagnosis:

Verified by :
walaah ghoniem

Printing Date : 22/01/2022 13:18:44

Page 1 of 2

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ملف الباثولوجي - 142/22 - 28908020103245 - أورام دار السلام - نفقة الدولة - 13/01/2022 - 19/01/2022 - 138551 - معمل الباثولوجي - المبادرة الرئاسية لصحة المرأة - 2022

Patient Name : الأرنؤاء / ولاء كمال حنفي على
Patient No. : 28908020103245
Path No. : 142/22
Sex : Female
Age : 32 Y
Referring Doc :

Request No. : 138551
Request Date : 13/01/2022
Verification Date : 19/01/2022
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

- Right breast mass; Right WLE and SLNs biopsy:
- Largest mass: Invasive duct carcinoma, grade II
 - Second mass: Mixed invasive duct carcinoma and mucoid carcinoma, grade II
 - Third mass: ductal carcinoma insitu.
 - Free margins
 - free SLNs: (0/6)
 - pT2(n) pNO pMx.

Prof. Dr. Aml Hareedy
Dr. Walaa Ghonaim.

Verified by:
walaa ghoniem

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Page 2 of 2

Printing By : marwa mohamed youss

Room No. 641 - First Floor Building No. 505 - Nile City - El-Dokki - Cairo
Phone: 02-26111111 - Fax: 02-26111111



Patient Name: Walaa Kamal Hanafy Hanafy Ali	Date : 05/01/2023
Exam : MAMMOGRAPHY BOTH	Age : 33 Year(s)
Medical ID: 55/2021	

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had right CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- No pathological right axillary lymph nodes.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Yours truly

Dr/Asmma Zelnhom, Msc

Signature Temp

05/01/2023 1:45 PM

Patient Name: Walaa Kamal Hanafy Hanafy Ali	Date : 25/03/2023
Exam : MRI BRAIN	Age : 33 Year(s)
Medical ID:	

MRI STUDY OF THE BRAIN

Technique of Examination:

- Axial T1, T2 & FLAIR WIs
- Sagittal T1 WIs
- No contrast was given

MR Findings:

- No intra cranial masses on non-contrast base.
- No cerebral parenchymal areas of abnormal signal.
- Normal grey / white matter differentiation.
- Normal size and configuration of the ventricular system.
- No shift of midline structures.
- No areas of fresh blood or blood degradation product signal.
- Normal appearance of the brain stem and cerebellum.
- No extra axial collections.

Opinion:

Normal MR study of the brain, for further evaluation by contrast enhanced study if clinically indicated.

Dr. Alamin Haider

Dr. Sherif Sameh, M.D.

Name: Wafaa Kamal Hanafy

Date: 26/12/2021

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

1. Chest:

- Right breast UOQ shows heterogeneously enhanced mass measuring about 24x21 mm.
- Right axillary level I and III prominent lymph nodes are seen the largest measures about 18x11 mm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal sizable lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUCD is seen slightly inferiorly displaced with small fluid collection.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No sizable lytic or sclerotic bone lesions.

OPINION:

- *Right breast heterogeneously enhanced mass.*
- *No CT evidence of pulmonary or hepatic metastatic deposits.*

Dr. Reham Anwar, MSc, FEBR

Dr. M. Goma, M.D.

Patient Name: الأستاذة / د. كمال حنلى حنلى
Patient No.: 28908020103245
Ph No.: 142/22
Sex: Female
Age: 32 Y
Referring Doc.:

Request No.: 138551
Request Date: 13/01/2022
Verification Date: 01/02/2022
Financial Category: اورام دار السلام - نفقة الدولة
Department: Out Patient

Registered by: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ER: Strong nuclear reaction in 98% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 98% of tumor cells (Allred score 8/8)
Her2: Weak incomplete membrane reaction in 10% of tumor cells. (score 1+), Negative
Ki 67: 60 % nuclear reaction

Diagnosis:
Right breast mass; Right WLE and SLNs biopsy:
Largest mass: Invasive duct carcinoma, grade II
Second mass: Mixed Invasive duct carcinoma and mucoid carcinoma, grade II
Third mass: ductal carcinoma insitu.
Free margins
SLNs: (0/6)
pNO pMx.
ER-1: Positive (8/8).
ER-2: Positive (8/8).
ER-2: Negative (score 1).
Ki67: Positive in 60% of tumor cells.

Menna Atef

Prof. Dr. Aml Hareedy
Dr. Walaa Ghonaim.

Verified by:
Rana fathy

Printing Date: 01/02/2022 11:07:03

Page 2 of 2

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Patient Name : الأستاذة / رلاء كمال خاتمي خنفر علي
Patient No. : 2890802010245
Path No. : 2965/21
Sex : Female
Age : 32 Y
Referring Doctor:

Request No. : 139917
Request Date : 01/12/2021
Verification Date : 20/12/2021
Financial Category : اورام دار السلام - فئة أشرطة
Department : Out Patient
IHC No. :

IMMUNOHISTOCHEMISTRY REPORT

Initial Data:
Right breast lesion.

Nature of the specimen:
Tru-cut biopsy.

Gross Examination:
Multiple grayish white and focally tan and brownish tissue cores, up to 1.5 cm, totally submitted.
Procedure: Tru-cut biopsy.
Localization: Right breast

Microscopic examination:
Histological type: Invasive duct carcinoma
Grade (differentiation + nuclear pleomorphism + mitosis): II
Ductal carcinoma in situ: Focal
Lymph node involvement: N/A
Resection Margins: N/A
Histologic response to therapy: N/A
Additional pathologic findings: N/A

Diagnosis:
Right breast lesion; Tru-cut biopsy:
Invasive duct carcinoma, nuclear grade II
C:
2: Strong nuclear reaction in 100% of tumour cells. Positive, score (8/8)
2: Strong nuclear reaction in 75% of tumour cells. Positive, score (6/8)
1+2: No membrane reaction in tumour cells. Negative, score 0
3+7: 22%

Dr. Amal Hareedy

Printing Date : 21/12/2021 13:10:01

Page 1 of 2

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Patient Name: Walaa Kamal Hanafy Hanafy Ali	Date : 08/07/2024
Exam : X-RAY CHEST	Age : 35 Year(s)
Medical ID: 55/2021	Pat. ID: 21122

Plain X-ray Chest (PA view) revealed:

- Clear both lung fields.
- Clear both costo-phrenic angles.
- Normal cardiac size & configuration.
- Normal thoracic cage

○ Signature Dr . Nermien Ramadan
○ 08/07/2024 12:27 PM



Patient Name: Walaa Kamal Hanafy Hanafy Ali	Date : 08/07/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 35 Year(s)
Medical ID: 55/2024	Pat. ID: 21122

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Enlarged in size (18cm at Rt MCL) bright parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Blad, 27: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

Average size homogenous.

Kidneys:

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites:

Urinary bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexae: no abnormalities could be detected, IUCD

insitu

Opinion:

Bright hepatomegaly likely fatty

Colonic gasous distention

Signature Dr. Mohamed Ramadan

PM 12:27 2024/07/08



Patient Name: Walaa Kamal Hanafy Hanafy	Date : 10/10/2022
Exam : C.T.BBRAIN WITH CONTRAST	Age : 33 Year(s)
Medical ID:	

**MULTI-SLICE CT SCAN OF THE BRAIN AFTER IV-CONTRAST
ADMINISTRATION: -**

Known case of breast cancer:

Findings:

- No focal cerebral areas of abnormal enhancement pattern.
- Preserved normal gray-white matter interface.
- No extra-axial or intra-cerebral areas of fresh blood densities.
- Normal extra-axial CSF spaces & basal skull cisterns.
- Normal size and configuration of the ventricular system.
- No midline shift.
- Unremarkable posterior fossa structures.

OPINION:

- Normal contrast enhanced CT scan of the brain.

Dr. Mark Adel

Dr. Amr Farouk, M.D.



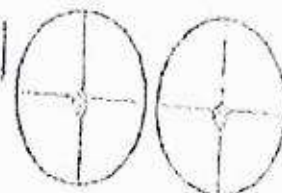
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MDT FORM

Name	WILLIAM J. H. S.
Age	30 years
MHN	C. 181 / 66
ID	S. A. G. H. C. I. F. E. L.
Co.	
morbidity	111877C-11



Strong Family History

Clinical Data

BS:	1DC 0.5	SM:
HC:	8/9	Mets Workup:
EN:	6/8	
PR:	22%	
HT:		
WT:		
T M M		
COMMENT:		
RT BCs, SNLB $\Rightarrow$ PT _{2m} No. mo. 1/6		
ER Lm O BC		

MDT OUTCOME 3: PLAN

intra- & young Age (< 35), multistage, 1A/2A
 15-Acylated chemotherapy ACK 4 cycle
 followed by PCRT + A-C + Zoladex

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
S. A. Ball			